STATEMENT OF DEFICIENCIES AND PLAN OF CORRECTION	(X1) PROVIDER/SUPPLIER/CLIA IDENTIFICATION NUMBER:	(X2) MULTIPLE CONSTRUCTION	(X3) DATE SURVEY COMPLETED
	0015955	A. BUILDING: _____ B. WING _____	C 02/18/2026

NAME OF PROVIDER OR SUPPLIER	STREET ADDRESS, CITY, STATE, ZIP CODE
WATERFORD AT COLBY (THE)	**1110 N DIVISION ST COLBY, WI 54421**

(X4) ID PREFIX TAG	SUMMARY STATEMENT OF DEFICIENCIES (EACH DEFICIENCY MUST BE PRECEDED BY FULL REGULATORY OR LSC IDENTIFYING INFORMATION)	ID PREFIX TAG	PROVIDER'S PLAN OF CORRECTION (EACH CORRECTIVE ACTION SHOULD BE CROSS-REFERENCED TO THE APPROPRIATE DEFICIENCY)	(X5) COMPLETE DATE
N 000	Initial Comments On 02/18/2026, Surveyor conducted 2 complaint investigations at The Waterford at Colby. One of the 2 complaints was substantiated. One deficiency was identified. The deficiency was a repeat deficiency. See Statement of Deficiency Y0HS11, dated 05/27/2021. Census: 44	N 000		
N 389	83.35(3)(d) Service plans updated annually or on changes Individual service plan review. Annually or when there is a change in a resident ' s needs, abilities or physical or mental condition, the individual service plan shall be reviewed and revised based on the assessment under sub. (1). All reviews of the individual service plan shall include input from the resident or legal representative, case manager, resident care staff, and other service providers as appropriate. The resident or resident ' s legal representative shall sign the individual service plan, acknowledging their involvement in, understanding of and agreement with the individual service plan. This Rule is not met as evidenced by: Based on record review and interview, the provider did not update the individual service plan (ISP) for 1 of 2 residents reviewed (Resident 1) when s/he had a change in condition. This is a repeat deficiency. See Statement of	N 389		

LABORATORY DIRECTOR'S OR PROVIDER/SUPPLIER REPRESENTATIVE'S SIGNATURE | TITLE | (X6) DATE

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N 389	Continued From page 1 Deficiency Y0HS11, dated 05/27/2021. Findings include: On 02/16/2025, the Department received a complaint that alleged staff were not offering a resident to use the bathroom. On 02/18/2025, Surveyor reviewed Resident 1's record. Resident 1 was admitted to the facility on 11/18/2024. S/he had multiple diagnoses including: acute respiratory failure with hypoxia (low oxygen), age related cognitive decline, anemia, Parkinson's disease, anxiety, assistance needed for personal care, constipation, and muscle weakness. Resident 1's ISP, dated 10/30/2025, read, in part: "TOILETING Needs/Details Full assistance Incontinence checks during waking hours Incontinence checks during nighttime hours Bedside commode Catheter-indwelling Adult briefs Notes: [Hospice] to provide cath (catheter) care and cath changes Objective/Plan the Care Team supports me maintaining healthy urine and bowel output and proper personal hygiene." On 12/14/2025, staff documented, in part: "As I was doing these cares, I noticed the reeking smell of BM (bowel movement) ... So I went to lift [his/her] shirt and my hand was covered in it. I washed my hands, lifted [him/her] in bed, and started to change [him/her] for bed. [S/he] had BM all the way down [his/her] leg and into	N 389		

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N 389	Continued From page 2	N 389		

[his/her] shoes, up [his/her] back ... and seeping out of [his/her] depends [sic] ... I noticed the outside pieces were crusty, so [s/he] had to have been sitting in it for a while ...[his/her] buttocks were very raw, red, and bleeding ..."

On 12/30/2025, Wellness Director (WD) A documented: "Hospice Removed [sic] residents [sic] cath on 12/29/2025.

On 02/18/2026 at approximately 2:45 p.m., Surveyor interviewed Activity Director (AD) B who was also the director for the unit Resident 1 resided on. Surveyor asked if Resident 1's toileting schedule changed after his/her catheter was removed. AD B told Surveyor Resident 1's toileting schedule was not changed after the catheter removal. AD B said Resident 1 would ring his/her call pendant and s/he was generally wet when staff go to change him/her. AD B said Resident 1 was always incontinent of bowel. Surveyor asked if staff ever assisted Resident 1 to use the bedside commode. AD B said s/he was not sure if Resident 1 ever used the commode.

On 02/18/2026 at 2:50 p.m., Surveyor interviewed Registered Nurse (RN) F on the phone. RN F was Resident 1's managed care organization (MCO) nurse. RN F told Surveyor Resident 1 had falls with fractures to both of his/her legs over a year ago and s/he had a urinary catheter placed at that time. RN F said Resident 1 was continent before catheter placement. RN F said they had a meeting with Resident 1 on 02/11/2026. S/he said s/he asked Resident 1 if s/he could tell when s/he needed to go to the bathroom. RN F said Resident 1 nodded his/her head yes to indicate s/he could tell when s/he needed to use the bathroom.

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N 389	Continued From page 3 On 02/18/2026 at 3:10 p.m., Surveyor interviewed RN H on the phone. RN H was Resident 1's hospice nurse. RN H said when s/he removed Resident 1's catheter, s/he spoke to staff that were working and asked them to put Resident 1 on a toileting schedule. RN H said s/he did not think staff were putting Resident 1 on the toilet at all for bowel movements while Resident 1 had the urinary catheter. RN H told Surveyor it was a dignity issue to put residents on the toilet even if they were incontinent. On 02/18/2026 at 3:15 p.m., Surveyor interviewed Family Member (FM) G on the phone. FM G said s/he was present at Resident 1's MCO meeting on 02/11/2026. FM G said they asked Resident 1 if staff took him/her to the bathroom and Resident 1 shook his/her head no. FM G said, "I don't know why they're not taking [him/her] to the bathroom." FM G said when Resident 1 was at a rehabilitation facility, s/he would use the bed pan. FM G said prior to Resident 1's catheter, s/he would call for help if s/he needed help to use the bathroom. FM G told Surveyor s/he did not think staff were putting Resident 1 on the toilet to have a bowel movement when s/he had the catheter. FM G told Surveyor, "[S/he] is declining, but [s/he] does know what is going on." FM G told Surveyor it was "very frustrating." On 02/18/2026 at 3:24 p.m., Surveyor interviewed Caregiver E. Surveyor asked if staff put Resident 1 on the toilet or commode. Caregiver E told Surveyor they did not. Caregiver E said Resident 1 was a check-and-change every 2 hours. On 02/18/2026 at 3:25 p.m., Surveyor interviewed Caregiver D. Surveyor asked if staff put Resident 1 on the toilet or commode. Caregiver D told	N 389		

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N 389	Continued From page 4 Surveyor they did not. Caregiver D said Resident 1 would ring when s/he was wet and staff changed him/her. The provider did not update Resident 1's ISP when s/he had an indwelling catheter removed. Resident 1's ISP was not revised to identify Resident 1's toileting needs and did not include services, frequency, and approaches staff were to provide to meet Resident 1's toileting needs.	N 389		